Goserelin 10.8mg Therapy- 12 weeks

INDICATIONS FOR USE:

INDICATION	ICD10	Regimen Code	HSE approved reimbursement status*
Treatment of locally advanced or metastatic hormone sensitive prostate cancer	C61	00477a	N/A

* This applies to post 2012 indications

TREATMENT:

Goserelin 10.8mg is injected subcutaneously into the anterior abdominal wall once every 12 weeks until disease progression or unacceptable toxicity.

Day	Drug	Dose	Route	Diluent & Rate	Cycle
1	Goserelin	10.8mg	SC	n/a	Every 12 weeks
Use extra care when administering goserelin to patients with a low BMI and/or who are receiving full anticoagulation medication					

ELIGIBILITY:

- Indication as above

CAUTIONS:

- Androgen deprivation therapy may prolong the QT interval. In patients with a history of or risk factors for QT prolongation and in patients receiving concomitant medicinal products that might prolong the QT interval, physicians should assess the benefit risk ratio including the potential for Torsade de pointes prior to initiating treatment.

EXCLUSIONS:

- Hypersensitivity to goserelin or any of the excipients

PRESCRIPTIVE AUTHORITY:

The treatment plan must be initiated by a Consultant with expertise in the treatment of prostate carcinoma.

TESTS:

Baseline tests:

- FBC, renal and liver profile
- Bone profile
- Blood glucose

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Regular tests:

- FBC, renal and liver profile
- Blood glucose and bone profile

Disease monitoring:
Disease monitoring should be in line with the patient's treatment plan and any other test/s as directed by the supervising Consultant.

DOSE MODIFICATIONS:

- No recommended dose modifications.

Table 1: Dose modification of goserelin in renal and hepatic impairment

Renal Impairment	Hepatic Impairment
No dose modification necessary	No dose modification necessary

SUPPORTIVE CARE:

EMETOGENIC POTENTIAL:

- As outlined in NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting - Available on NCCP website

Goserelin: Minimal (**Refer to local policy)**

- **For information:**
  Within NCIS regimens, antiemetics have been standardised by Medical Oncologists and Haemato-oncologists. Information is available in the following documents:
- NCCP Supportive Care Antiemetic Medicines for Inclusion in NCIS (Medical Oncology) - Available on NCCP website
- NCCP Supportive Care Antiemetic Medicines for Inclusion in NCIS (Haemato-oncology) - Available on NCCP website

PREMEDICATIONS: None

OTHER SUPPORTIVE CARE:

Calcium and vitamin D supplementation **(Refer to local policy)**

ADVERSE EFFECTS

- Please refer to the relevant Summary of Product Characteristics (SmPC) for details.

DRUG INTERACTIONS:

- Current drug interaction databases should be consulted for more information.

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REFERENCES:

1. Loblaw DA, Virgo KS, Nam R, et al. Initial hormonal management of androgen-sensitive metastatic, recurrent, or progressive prostate cancer: update of an American Society of Clinical Oncology practice guideline. J Clin Oncol 2006;25(12):1596-1605
2. Zoladex ® Summary of Product Characteristics Accessed July 2025. Last updated 22/02/2023. Available at: https://www.medicines.ie/medicines/zoladex-la-10-8mg-34372/spc

Version	Date	Amendment	Approved By
1	30/05/2018		Prof Maccon Keane
2	10/06/2020	Regimen review	Prof Maccon Keane
3	30/07/2025	Reviewed. ATC code removed. Cautions section added. Regimen updated in line with NCCP standardisation.	Prof Maccon Keane

Comments and feedback welcome at oncologydrugs@cancercontrol.ie.

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